PRACTICE EXAM 04 / EXAM SET 01

Domain 4: Patient Care Coordination and Education

12 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | Preventive Care and Care Gaps

> **SCENARIO** During previsit planning, a medical assistant sees a colorectal screening reminder in the electronic health record. The patient reports having a colonoscopy at an outside facility last year, but the report is absent. The provider will review screening needs at tomorrow's visit.

Which action best prepares the record for the provider's review?

A. Mark the screening complete using the patient's estimate of the procedure date.

B. Keep the reminder active without recording the reported outside procedure.

C. Document the patient's report and request the outside procedure report through the approved process.

D. Enter a new screening due date using the usual interval for colonoscopy.

Q02 | Referrals and Care Transitions

> **SCENARIO** A patient confirms attending an ordered cardiology consultation two weeks ago. The referral tracker still shows an open referral, and the primary care record contains no consultation report. The patient's next primary care visit is in two days.

What should the medical assistant do to move this referral toward closure?

A. Request the consultation report, route it for provider review, and track any required follow-up.

B. Close the referral after documenting that the patient attended the appointment.

C. Ask the patient to summarize the recommendations and use that summary as the consultation report.

D. Send a duplicate cardiology referral so a new consultation can generate another report.

Q03 | Patient Education and Self-Management Support

SCENARIO A provider has ordered a home blood pressure monitor and approved a device-specific teaching checklist. After the assistant demonstrates the checklist steps, the patient says, "I understand." The assistant needs to evaluate whether the instruction was effective.

Which finding provides the best evidence that the patient can perform the task?

A. The patient states that the written instructions look familiar.
B. The patient agrees to read the monitor booklet before using it.
C. The patient identifies the monitor's brand and model correctly.
D. The patient performs the measurement steps correctly using the checklist.

Q04 | Community Resources and Access Barriers

SCENARIO A patient has missed two ordered physical therapy appointments because a wheelchair-accessible ride is unavailable. The patient wants to attend and agrees to assistance. A clinic resource directory lists an accessible ride program, but reservation requirements have not been confirmed.

Which coordination action best addresses the stated barrier?

A. Move therapy to a later month and close the transportation task until the patient calls again.
B. Contact the ride program with the patient to confirm access and booking needs, then verify the ride arrangement.
C. Reschedule the same appointment time and remind the patient to arrange transportation.
D. Give the patient a list of general taxi companies and mark the barrier resolved.

Q05 | Referrals and Care Transitions

SCENARIO During a post-discharge call, a patient reports that the hospital paperwork and a prescription bottle show different doses of the same medication. The patient asks which dose to take. Clinic policy directs medication discrepancies to the nurse or provider for prompt clarification.

Which response stays within the assistant's role?

A. Use the dose on the bottle because it was dispensed by a pharmacy.
B. Use the discharge dose because hospital instructions are more recent.
C. Recommend a dose between the two until the follow-up appointment.
D. Document both instructions and arrange prompt clarification by the nurse or provider under clinic policy.

Q06 | Patient Education and Self-Management Support

SCENARIO A provider's written nutrition plan asks a patient to replace one sugar-sweetened drink each day with water. The patient says, "I can do that, but my workplace has only a soda vending machine." The assistant is reinforcing this established goal.

Which action best supports the patient's self-management?

A. Help the patient plan how to bring water to work and agree on a way to track the daily substitution.
B. Provide a beverage handout and defer planning for water access until the next follow-up visit.
C. Ask the patient to repeat the goal at each visit without discussing workplace arrangements.
D. Suggest waiting to begin the goal until the workplace installs a drinking fountain.

Q07 | Community Resources and Access Barriers

SCENARIO A patient reports running out of food before the end of each month and being unable to obtain the foods in a dietitian's existing plan. The patient would like help. The clinic has a resource navigator who can assess food-program eligibility and local food availability.

Which action best connects the patient with useful support?

A. Provide a low-cost grocery list and document the food-access issue as resolved.
B. Suggest buying foods in bulk and defer resource assistance until the next visit.
C. Connect the patient with the resource navigator for food assistance and track whether contact occurs.
D. Document that the patient prefers other foods and leave the plan unchanged without follow-up.

Q08 | Preventive Care and Care Gaps

SCENARIO A patient has not completed a provider-recommended screening mammogram. During outreach, the patient says the scheduling call was confusing and they remain unsure how to proceed. The patient has not said that they refuse the screening.

Which entry and follow-up best support accurate care-gap tracking?

A. Document the scheduling difficulty, offer scheduling assistance, and retain the task for follow-up.
B. Document screening refusal and close the task because the test remains incomplete.
C. Mark the screening complete because outreach has now been performed.
D. Remove the recommendation because the patient has not scheduled it independently.

Q09 | Patient Education and Self-Management Support

> **SCENARIO** During a virtual visit, an assistant reinforces the provider's written home-care plan. The patient can access the handout but loses track when all six instructions are reviewed together. The connection is clear, and the patient wants to continue.

Which teaching adjustment best addresses the observed difficulty?

A. Read all six instructions more quickly so the patient hears them as a single message.
B. Review a small group of instructions, ask the patient to explain that part in their own words, and then continue.
C. Send a longer handout and end the teaching without checking understanding.
D. Ask whether the patient understands after repeating the full list in the same way.

Q10 | Preventive Care and Care Gaps

> **SCENARIO** Before a child's visit, the assistant reviews an outside immunization record. The electronic health record lists two doses of the same vaccine on the same date, but the outside record shows only one administration. The provider has asked the assistant to prepare a verified immunization history before reviewing today's needs.

How should the assistant handle the discrepancy?

A. Count both electronic entries as separate doses when preparing the history.
B. Delete one entry immediately because same-day entries must be duplicates.
C. Use the discrepancy to choose which vaccine dose the child should receive today.
D. Flag the conflicting entries, verify the source details, and route the discrepancy through the clinic's correction process.

Q11 | Community Resources and Access Barriers

> **SCENARIO** A patient who cares for a spouse with memory impairment says they cannot attend their own follow-up visits because the spouse cannot be left alone. The patient requests information about supervised daytime support. The clinic directory includes an adult day services program and a respite resource coordinator.

Which referral best matches this barrier?

A. A caregiver education group to teach the patient more about memory impairment.
B. A meal delivery program to reduce the time the patient spends preparing food.
C. The respite coordinator to explore appropriate daytime supervision and availability before the next visit.
D. A telephone check-in service to call the spouse while the patient attends appointments.

Q12 | Referrals and Care Transitions

> **SCENARIO** A patient is scheduled for a primary care follow-up after an emergency department visit. A discharge summary identifies a pending urine culture, but no result is in the clinic record. The clinic workflow assigns the follow-up nurse to track pending transition results and the provider to interpret them.

Which handoff best supports continuity for this visit?

A. File the discharge summary and omit the pending culture because it has no final result yet.

B. Alert the follow-up nurse to the pending culture, record the result source and status, and track routing for provider review.

C. Tell the patient that the culture was probably normal because no result has been received.

D. Ask the laboratory for a preliminary finding and use it to recommend a medication change.

ANSWER KEY & EXPLANATIONS

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01 C	02 A	03 D	04 B	05 D	06 A
07 C	08 A	09 B	10 D	11 C	12 B

DETAILED EXPLANATIONS

Q01 | Correct answer: C

Document the patient's report and request the outside procedure report through the approved process.

Recording the patient-reported history and seeking the actual report gives the provider information needed to determine screening status. A reported procedure should not be treated as a verified result or used by the assistant to establish a new screening interval.

Q02 | Correct answer: A

Request the consultation report, route it for provider review, and track any required follow-up.

Attendance alone does not establish that the referring team received and acted on the specialist's recommendations. Obtaining the report, routing it for clinical review, and tracking the resulting follow-up supports continuity without asking the assistant to interpret the findings.

Q03 | Correct answer: D

The patient performs the measurement steps correctly using the checklist.

A return demonstration directly shows whether the patient can carry out the taught procedure. Agreement or recognition of the device does not demonstrate correct technique; the assistant should address any observed errors and recheck performance.

Q04 | Correct answer: B

Contact the ride program with the patient to confirm access and booking needs, then verify the ride arrangement.

The barrier is access to a suitable ride, so coordination must establish whether the resource can meet the patient's mobility and scheduling needs. A specific booking plan and follow-up are more useful than another reminder or an unverified list of transportation options.

Q05 | Correct answer: D

Document both instructions and arrange prompt clarification by the nurse or provider under clinic policy.

The discrepancy needs clinical clarification, and both sources should be available to the licensed team member resolving it. Selecting either dose based only on its source or apparent date would make an unauthorized medication decision.

Q06 | Correct answer: A

Help the patient plan how to bring water to work and agree on a way to track the daily substitution.

A practical plan for obtaining water addresses the patient's stated obstacle while preserving the provider's goal. Tracking the substitution supports follow-up; prescribing a different diet or simply repeating the instruction does not address the implementation problem.

Q07 | Correct answer: C

Connect the patient with the resource navigator for food assistance and track whether contact occurs.

Food insecurity is a resource barrier that needs a specific assistance connection and follow-up. The assistant can facilitate that connection without revising the dietitian's clinical plan or assuming the patient is unwilling to follow it.

Q08 | Correct answer: A

Document the scheduling difficulty, offer scheduling assistance, and retain the task for follow-up.

The record should reflect the barrier the patient actually describes. Scheduling assistance and continued tracking address that barrier; an incomplete screening should not be relabeled as refusal or completion merely to close the task.

Q09 | Correct answer: B

Review a small group of instructions, ask the patient to explain that part in their own words, and then continue.

Breaking information into manageable segments and checking understanding after each segment allows errors to be identified and corrected before moving on. Repeating the same long presentation or asking only for a yes-or-no response does not establish understanding.

Q10 | Correct answer: D

Flag the conflicting entries, verify the source details, and route the discrepancy through the clinic's correction process.

Conflicting documentation should be verified and corrected through the approved process before it is treated as an accurate history. The assistant should neither count uncertain entries as distinct administrations nor independently decide the child's vaccine needs.

Q11 | Correct answer: C

The respite coordinator to explore appropriate daytime supervision and availability before the next visit.

The patient needs a workable supervision resource to make attendance possible. A respite coordinator can help identify suitable daytime options and availability; a telephone check-in does not provide the supervision described as necessary.

Q12 | Correct answer: B

Alert the follow-up nurse to the pending culture, record the result source and status, and track routing for provider review.

A reliable transition handoff identifies unfinished testing and connects it to the team member responsible for tracking and clinical review. A missing result is not evidence of a normal result, and the assistant must not use laboratory findings to select treatment.

End of Practice Exam